

1. CLINICAL OVERVIEW

Patient presents with **Glioblastoma Multiforme** (IDH-Wildtype) Clinical-molecular profile indicates a **HIGH RISK** classification.

PATIENT NAME	MRN	RISK LEVEL
Jane Doe	MRN-20240087	HIGH
GENDER / AGE	DIAGNOSIS DATE	WHO GRADE
Female / 54	12 Jan 2024	Grade 4
DIAGNOSIS	SUBTYPE	STAGE
Glioblastoma Multiforme	IDH-Wildtype	IV

2. PRIMARY CLINICAL RECOMMENDATION

STUPP PROTOCOL — CONCURRENT TEMOZOLOMIDE + RT (60 GY/30 FX),
FOLLOWED BY ADJUVANT TMZ × 6 CYCLES

Clinical Rationale & Evidence Basis

- IDH-wildtype GBM with MGMT methylation confirmed — TMZ sensitivity predicted.
- KPS 80 supports aggressive standard-of-care protocol.
- No contraindications to alkylating agents on CBC review.
- EGFR amplification detected; consider erlotinib if recurrence.

3. GENOMIC & MOLECULAR LANDSCAPE

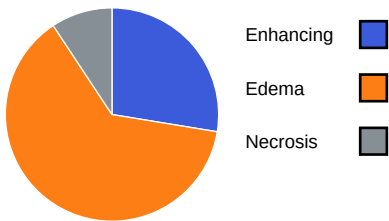
TOTAL VARIANTS	ACTIONABLE MUTATIONS	VUS
1,247	14	83
TMB	MSI STATUS	
8.4 mut/Mb	MSS	

GENE	STATUS	CLINICAL IMPACT	TIER
EGFR	Amplified	Potential erlotinib sensitivity	Tier I
PTEN	Deleted	PI3K/AKT pathway activation	Tier I
MGMT	Methylated	Improved TMZ response probability	Tier I
TP53	Mutated	Tumour suppressor loss	Tier II
CDKN2A	Deleted	Cell-cycle checkpoint dysregulation	Tier II

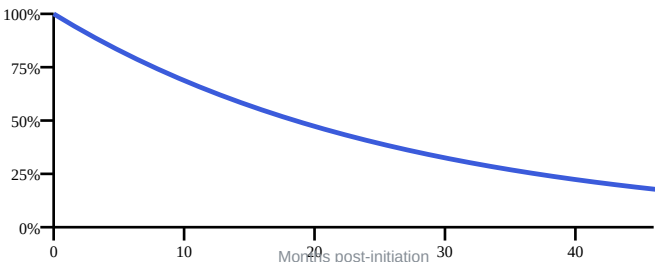
4. IMAGING & VOLUMETRIC ANALYTICS

CORE VOLUME (cc)	EDEMA VOLUME (cc)	NECROTIC %
18.4	42.1	6.2
SPHERICITY	MAX DIAMETER (mm)	KPS SCORE
0.71	38	80
ECOG SCORE		
1		

Volumetric Distribution



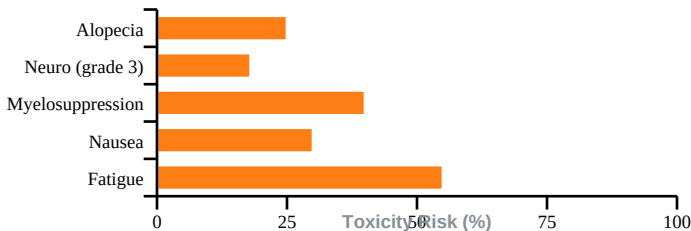
Estimated Survival Projection



5. ADVANCED OUTCOME PROJECTIONS

MEDIAN OS	MEDIAN PFS	RESPONSE RATE
18.5 Months	7.2 Months	42%
5-YR SURVIVAL		
9.8%		

Therapeutic Toxicity Profile



6. PATIENT-REPORTED OUTCOMES

30-Day Protocol Adherence: 86.0%

Recent Clinical Alerts

- [HIGH] Platelet count < 100 k/μL — hold TMZ, repeat CBC in 48 h.
- [MEDIUM] Patient reported grade-2 fatigue; supportive care initiated.

7. IMMEDIATE CLINICAL ACTIONS

1. Initiate Stupp protocol within 7 days of surgical confirmation.
2. Arrange simulation CT for radiotherapy planning.
3. Monitor CBC weekly; hold TMZ if ANC < $1.5 \times 10^9/L$.
4. Neurosurgery review at 6-week post-op MRI.